

How to take over management of Google reviews?

To take up control of the google reviews for the practice, you don't need the previous management company's login/password.

For a physical-location GP practice, the normal route is to request access/ownership in Google Business Profile, then verify the location.

Use a practice-controlled Google account (not an agency or management company one), then:

1. Sign in to a Google account the practice will keep long-term (ideally a shared role mailbox with 2FA).
2. Go to the add/claim flow and select the existing listing: <https://business.google.com/add>
 - Enter the practice name + address
 - Pick the existing listing from the results (it appears as New Bank Health Centre as you type)
 - If you see "Someone else may manage this Business Profile", choose **Request access** and submit the form: <https://support.google.com/business/answer/4566671?hl=en-GB>
3. What happens next
 - The current owner gets an email and has **3 days** to respond.
 - If they approve, you'll get an email and you're in.
 - If there's no response after 3 days, you may be offered a **Verify / Claim** option from the email ("View request" → "Verify") or inside the profile. Complete whatever verification method Google offers for that listing. (Google doesn't always offer the "claim after 3 days" path; if it doesn't appear, you'll need to go through support from within the Business Profile UI.)
4. If the listing is unclaimed/unverified
 - It's often simpler: claim it and verify it directly using Google's "Add or claim" flow: <https://support.google.com/business/answer/2911778?hl=en>

After access is sorted, lock it down so this doesn't happen again:

- Set the **primary owner** to a practice-controlled account.
- Add GTD Healthcare (or whoever's operationally running comms) as **Manager** (or Owner only if genuinely needed).
- Add at least one backup practice **Owner**.
- Expect some admin actions to be restricted for **7 days** for newly added owners/managers: <https://support.google.com/business/answer/3403100?hl=en-GB>

How to actually reply to reviews?

Reading reviews is a great way to spot opportunities for the practice to improve, and although reviewers do sometimes get push notifications for official replies to their reviews (so you can reach *some* lost or excluded patients), replying to reviews is as much for the next patient reading them, not just the person who wrote them.

The job is to show the practice takes feedback seriously, can own mistakes without getting defensive, and has a clear route for putting things right. If you're reading a negative review and your first impulse is to explain why the patient is wrong, stop. That might feel satisfying for a minute, but it makes the practice look brittle and uninterested in fixing anything.

For a positive review:

Thank them → feedback to the person/team responsible → \o/

For a negative review:

Acknowledge → apologise (when it's appropriate) → invite offline resolution → commit to learning/acting.

A plain apology is often the fastest way to lower the temperature, even if you think the situation was "procedurally correct". "I'm sorry you had this experience" or "I'm sorry you feel this way about..." or "Actually, our process is..." are all pretty common responses from practices, because it isn't admitting fault in a legal sense, it's 'safe', and it's easy to believe that that protects the practice, but all that really happens is the patient feels devalued and even alienated, and if they're in an excluded group who literally *can't* follow your 'well, actually', they end up even more lost.

A better pattern:

- name the problem they described in plain words
- accept responsibility for the experience of using the service
- say what you're doing next (or what you've already changed if it was easy to track down a cause)
- invite them to a private channel to resolve *their* case, without asking for details in public

Examples that don't sound like a template:

- "Sorry you couldn't get through to us and didn't get a call back when you expected. That isn't acceptable. If you email [address]/call [number] and mark it for the practice manager, we'll look into what happened and try to put it right."
- "I'm sorry your prescription was delayed and you were left chasing. We're reviewing how requests are tracked so patients aren't bounced between steps. Please contact us via [channel] so we can sort the specific issue privately."
- "Sorry you were spoken to like that. That's not the standard we expect. We'll raise this with the team and we'd like to understand what happened — please contact [channel] so we can follow up."

Keep replies from turning into patient-blame:

- Don't write "you should have..." or "our process is..." unless you're using it to fix confusion ("we realise our message about X wasn't clear; we're changing it"), not to win the argument.
- If you can't point to an action (already taken, in progress, or queued with an owner), the reply probably shouldn't go out yet. Log it, assign it, then respond, or at least ask for more information, so that patient has a chance to reconnect.

The objective isn't to be "nice" in public. It's to be competent, not just to respond, but to be responsive: We understood the complaint, we own the failure, we have/want a route to resolution, and we'll change something so it happens less.

The hidden objective is operational. Every negative review is a free signal about where patients are getting hurt: access, reception tone, call-backs, prescription delays, appointment triage,

unclear messaging, long waits, cancellations, continuity, whatever. Treat each one as an incident report: tag it, summarise the issue in a sentence, and make sure it lands somewhere that leads to action (front desk lead, practice manager, clinical lead, PPG themes, an issues log). If you can fix it immediately, do. If you can't, raise it, track it, and follow up internally.

A reply that doesn't trigger any change is just theatre, and people can tell because the same few issues have cropped up for this practice for years, and now the review rates are so low, people don't even bother complaining about a lot of these issues in detail, they just dump "terrible practice" and never come back.

Don't fall into the two lazy modes: only replying to praise ("thanks for your kind words") or replying to criticism with policy excerpts and patient-blame. The first creates confirmation bias and looks like you only engage when it's easy; the second confirms the reviewer's complaint and invites pile-ons.

A good response is reactive and responsive: it de-escalates, offers a route to resolution, and quietly demonstrates the practice is learning. Over time, that's what actually moves the score, not the volume of replies.